DISCHARGE SUMMARY

Patient: Jane Roe. Date of birth: 1958-06-14. Admitted: 2026-03-02. Discharged: 2026-03-09. Attending physician: Dr. Alan Whitfield, Department of Internal Medicine.

Reason for admission

The patient was admitted with chest pain radiating to the left arm that began two hours before arrival. She described the pain as pressure-like, rated seven out of ten, and associated with mild nausea and sweating. An ECG and a serial troponin series ruled out an acute myocardial infarction.

Past medical history

Type 2 diabetes mellitus, diagnosed in 2019 and treated with metformin. Hypertension controlled with lisinopril 10 mg daily. Hyperlipidemia managed with atorvastatin. No prior surgeries other than an uncomplicated appendectomy in 1984.

Social history

Retired schoolteacher. Never smoked. Drinks one glass of wine on weekends. Walks thirty minutes most days and lives with her husband, who is her primary support.

Nursing notes

Vital signs were recorded every four hours. The patient slept well on the second night and reported that the chest pressure had settled completely. She tolerated a regular cardiac diet and drank at least two liters of fluid per day. Skin was intact and the intravenous site remained clean and dry throughout the stay.

Physical examination

The patient was alert, oriented and in no acute distress. The cardiac examination revealed a regular rate and rhythm with no murmurs, rubs or gallops. Lungs were clear to auscultation bilaterally. The abdomen was soft and non-tender. There was no peripheral edema and distal pulses were symmetric.

Family history

Her father had a myocardial infarction at the age of 61 and her mother has hypertension. One brother has type 2 diabetes. There is no known family history of sudden cardiac death, cardiomyopathy or inherited arrhythmias.

Review of systems

The patient denied fever, chills, cough, abdominal pain, headache and recent weight change. She reported occasional mild fatigue over the past month. No orthopnea, no paroxysmal nocturnal dyspnea and no history of syncope were reported.

HOSPITAL COURSE

On arrival the patient was hemodynamically stable with a blood pressure of 148 over 86 and a heart rate of 82 beats per minute. Oxygen saturation was 97 percent on room air. She was started on aspirin and monitored on telemetry for the first twenty-four hours.

Serial troponin values at zero, three and six hours remained within normal limits. A resting echocardiogram showed preserved left ventricular function with an ejection fraction of 60 percent and no wall motion abnormalities.

A stress test on the fourth hospital day demonstrated a small reversible defect in the inferior wall. Cardiology recommended a coronary angiogram, which showed a single 60 percent stenosis of the right coronary artery that was managed medically.

Complications

There were no in-hospital complications. A transient rise in blood glucose to 240 mg/dL on the second day resolved after a temporary adjustment of the insulin sliding scale.

Consultation: cardiology

Cardiology reviewed the electrocardiogram, the echocardiogram and the stress test results. The consultant agreed that the presentation was consistent with stable angina and recommended guideline-directed medical therapy, risk factor modification and a structured exercise program.

Consultation: nutrition

A dietitian reviewed the patient’s eating habits and provided written guidance on portion sizes, carbohydrate counting and sodium reduction. The patient agreed to keep a food diary for the next month and to bring it to her next appointment.

Consultation: pharmacy

A clinical pharmacist reconciled the home medication list with the discharge orders, confirmed that there were no duplicate therapies and reviewed the correct use of sublingual nitroglycerin, including storage, dosing and when to call for help.

Functional status

At discharge the patient was independent in all activities of daily living and walked the length of the unit without assistance or symptoms. She climbed one flight of stairs with no chest discomfort and no shortness of breath.

Care coordination

The discharge plan was discussed with the patient, her husband and the primary care office. Transportation home was arranged by the family. The cardiac rehabilitation program will contact the patient by telephone within five business days to schedule an intake visit.

MEDICATIONS AT DISCHARGE

- Metformin 500 mg twice daily with meals.
- Lisinopril 10 mg once daily in the morning.
- Atorvastatin 40 mg nightly, increased from 20 mg.
- Aspirin 81 mg once daily.
- Metoprolol succinate 25 mg once daily, newly started.
- Nitroglycerin 0.4 mg sublingual as needed for chest pain.

Allergies

Penicillin causes a rash. No other known drug allergies. The patient carries a wallet card listing the reaction.

Medications stopped

Ibuprofen was stopped because of its interaction with lisinopril and the risk of kidney injury. The patient was advised to use acetaminophen for minor aches.

Immunizations

Influenza vaccination for the current season was administered during the admission. Pneumococcal vaccination is up to date. The patient was advised to review her tetanus and shingles vaccination status with her primary care physician.

Risk factor summary

Modifiable risk factors include suboptimal glycemic control, elevated LDL cholesterol and a sedentary period over the winter months. Non-modifiable factors include age, sex and a family history of premature coronary disease. Blood pressure has been at goal on the current regimen.

Patient goals

The patient stated that her main goals are to return to her garden, to travel to visit her grandchildren in the spring and to avoid another hospital stay. These goals were incorporated into the rehabilitation plan and the follow-up schedule.

Documentation notes

This summary was prepared from the inpatient record, including the admission note, daily progress notes, consultation reports, laboratory results and imaging reports. Where values are given they are the most recent available at the time of discharge.

Family history

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LABORATORY RESULTS

- Hemoglobin A1c: 7.4 percent, above the target of 7.0 percent.
- Fasting glucose: 142 mg/dL.
- Creatinine: 0.9 mg/dL with an estimated glomerular filtration rate of 78.
- LDL cholesterol: 118 mg/dL before the statin dose was increased.
- Potassium: 4.2 mmol/L. Sodium: 139 mmol/L.
- Troponin I: below 0.01 ng/mL at all three measurements.

Imaging

Chest radiograph showed clear lung fields and a normal cardiac silhouette. Coronary angiography confirmed a single 60 percent stenosis of the right coronary artery without flow limitation.

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ASSESSMENT AND PLAN

Chest pain with a negative myocardial infarction work-up and moderate single-vessel coronary disease. The pain is most consistent with stable angina. Optimal medical therapy was started and cardiac risk factors were addressed.

- Continue aspirin, atorvastatin and metoprolol indefinitely.
- Intensify diabetes management; the primary care physician will consider adding an SGLT2 inhibitor.
- Target blood pressure below 130 over 80.
- Enroll in the outpatient cardiac rehabilitation program.

Diet and activity

Follow a Mediterranean-style diet low in saturated fat and added sugar. Resume light walking immediately and increase gradually. Avoid heavy lifting for two weeks.

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FOLLOW UP

Follow up with the primary care physician in two weeks to review blood pressure and glucose logs. Repeat the HbA1c test in three months. Cardiology clinic visit is scheduled in six weeks.

Return precautions

Return to the emergency department if chest pain recurs, lasts longer than fifteen minutes or does not respond to one dose of nitroglycerin, or if shortness of breath, fainting or palpitations develop.

Patient education

The patient and her husband received written instructions and a medication schedule. They verbalized understanding and were able to repeat the warning signs of a heart attack.

Signed electronically by Dr. Alan Whitfield on 2026-03-09.

Communication

Results of all tests completed during the admission were reviewed with the patient before discharge. A copy of this summary, the medication schedule and the cardiac rehabilitation brochure were given to the patient and sent to the primary care physician and the referring cardiologist.

Nursing notes

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